

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 13

Age: 55 years old | Sex: Male

MRN: 000-TEST-13

Admission Date: [Current Date - 3 days] | Discharge Date: [Current Date]

Attending Physician: Dr. R. Mitchell, Internal Medicine

****PRIMARY DIAGNOSIS & COMORBIDITIES****

- COPD exacerbation, mild-to-moderate severity (GOLD Stage II)
- Type 2 Diabetes Mellitus
- Hyperlipidemia
- Hypertension

****HOSPITAL COURSE****

Patient 13 presented to Metro Medical Center via ED with acute-onset dyspnea, productive cough × 5 days, and increased sputum purulence. Baseline FEV1/FVC ratio documented at 58% (post-bronchodilator). SpO2 on admission 88% on room air; improved to 94–96% with supplemental O2 (2L NC). Initial CXR showed mild hyperinflation without acute infiltrate. Patient received IV methylprednisolone 125 mg q.6h × 48 hrs, then transitioned to PO prednisone taper. Albuterol nebulizer treatments q.4h initially, then q.6h as respiratory status improved. ABG on day 2: pH 7.38, pCO2 44, pO2 76 (on 2L). Patient remained hemodynamically stable; troponin × 1 negative. Diabetes management continued with home regimen; capillary glucose ranged 118–167 mg/dL throughout admission. No acute coronary syndrome or pneumonia identified. Discharged on hospital day 3 with significant clinical improvement: SpO2 96% on RA at rest, mild exertional DOE, ambulating independently

without difficulty.

****MEDICATIONS AT DISCHARGE****

| Medication | Dose | Frequency | Duration |

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| Tiotropium (LAMA) | 18 mcg | Once daily (q.d.) | Ongoing |

| Formoterol/Budesonide (LABA/ICS combination) | 80/4.5 mcg | 2 inhalations BID | Ongoing |

| Prednisone (taper) | 40 mg → 30 mg → 20 mg → 10 mg | Daily × 4 days then d/c | 4 days |

| Albuterol (rescue inhaler) | 90 mcg | 1–2 puffs q.4–6h PRN | Ongoing |

| Atorvastatin | 20 mg | q.d. at bedtime | Ongoing |

| Lisinopril | 10 mg | q.d. | Ongoing |

| Metformin XR | 500 mg | BID with meals | Ongoing |

****ACTIVITY & MOBILITY****

No weight-bearing restrictions. Patient may resume normal activities as tolerated. Encourage gradual increase in ambulation. Avoid strenuous exertion and heavy lifting × 2 weeks. No smoking.

****DIET****

No specific dietary restrictions. Continue diabetic diet (low glycemic index, appropriate portion control).

****FOLLOW-UP APPOINTMENTS****

- Pulmonology clinic: In 2 weeks (with spirometry & FEV1/FVC reassessment)
- Primary Care/Internal Medicine: In 1 week
- Endocrinology (optional): In 4–6 weeks for diabetes management review

****RED FLAG SYMPTOMS — SEEK IMMEDIATE MEDICAL ATTENTION IF:****

- Severe dyspnea or SOB at rest
- SpO₂ < 88% (